

Management of Acute Scrotal Pain & Suspected Testicular Torsion Procedure

TABLE OF CONTENTS

[Precautions](#)

[Procedure](#)

[Adult \(16 years or older\) patient in hours](#)

[Adult \(16 years or older\) patient out of hours](#)

[Paediatric \(less than 16 years of age\) patient in hours](#)

[Paediatric \(less than 16 years of age\) patient out of hours](#)

[Appendix 1: Emergency Department referral flow charts](#)

Presentation to:

[Casey Emergency Department](#)

[Clayton Emergency Department](#)

[Dandenong Emergency Department](#)

[VHH Emergency Department](#)

AUTHOR/S

This procedure has been developed by Monash Health General Surgery, Urology and Paediatric Surgery Units.

TARGET AUDIENCE and SETTING

Medical and nursing staff of all Monash Health sites.

PURPOSE

To optimise the timely management of suspected testicular torsion and comply with Royal Australasian College of Surgeons consensus guidelines.

DEFINITIONS

Acute scrotum = acute onset of scrotal pain for which testicular torsion is a likely or suspected diagnosis.

PRECAUTIONS

Seek surgical opinion immediately.

If relevant Registrar is not available, escalate to Consultant on-call.

Adult patients are to be managed at the site of presentation, and transfer to Monash Children's Hospital must be expedited for patients under the age of 16. Treatment delays resulting from transfer greatly increases the risk of testicular infarction if torsion is present.

STANDARD REQUIREMENTS

PROMPT Doc No: SNH0033332 v11.0		
Date loaded on PROMPT: 07/03/2013	Page 1 of 7	Review By: 22/10/2027
Version Changed: 22/03/2024	Document uncontrolled when downloaded.	Last Reviewed Date: 22/03/2024

Management of Acute Scrotal Pain & Suspected Testicular Torsion Procedure

When undertaking any clinical interaction with a patient, staff are expected to:

- Perform routine hand hygiene. Refer to the [Hand Hygiene Procedure](#).
- Introduce themselves to the Patient and Carer/ Family if in attendance.
- Check patient identification. Refer to the [Patient Identification Procedure](#).
- Obtain consent as per the [Consent to Medical Treatment Procedure](#).
- Keep the patient/carer informed and involve them in decision making.
- Document interaction in the electronic medical record or health record using black pen; including date, time, signature and designation.

PROCEDURE

1. Any patient with scrotal pain, on presentation to ED, is to be **assigned Australian Triage Category 2**: Patient to be seen within 10 minutes by ED Consultant or Senior Registrar.
2. Keep patient fasted until definitive management is determined.
3. **Do not perform any investigations** (e.g., urine microscopy, ultrasound) **prior to surgical review**.
4. ED senior doctor to contact relevant on-call service (below) for **review within 30 minutes**:
 - Patient 16+ years; in hours (0700 – 1800)
 - Casey → Urology
 - Clayton → ASU (General Surgery)
 - Dandenong → ASU (General Surgery)
 - Victorian Heart Hospital (VHH) → urgent transfer to Clayton ED via contact with ASU (General Surgery)
 - Patient 16+ years; after hours/weekends
 - All sites → ASU (General Surgery) Registrar → Urology On-Call
 - VHH → urgent transfer to Clayton ED via contact with ASU (General Surgery)
 - Patient <16 years; in hours (0700-1800)
 - Clayton → Paediatric Surgery
 - Casey → Urology Registrar/Fellow for assessment, then refer to Paediatric Surgery and transfer to Clayton after clinical diagnosis is confirmed. If there is **any delay to transfer, escalate to Urology Consultant on-call**.
 - Dandenong → ASU (General Surgery) for assessment, then refer to Paediatric Surgery and transfer to Clayton after clinical diagnosis is confirmed. If there is **any delay to transfer, escalate to ASU Consultant on-call**.
 - VHH → urgent transfer to Monash Children's ED via contact with Paediatric Surgery
 - Patient <16 years; after hours/weekends
 - Clayton → Paediatric Surgery
 - Casey or Dandenong → ASU (General Surgery) for assessment, then refer to Paediatric Surgery and transfer to Clayton after clinical diagnosis is confirmed. If there is **any delay to transfer, escalate to ASU Consultant on-call**.

Management of Acute Scrotal Pain & Suspected Testicular Torsion Procedure

- VHH → urgent transfer to Monash Children's ED via contact with Paediatric Surgery
- 5. For adult patients, if surgical intervention is required, it is to be arranged at the site of presentation as **Category 2: Urgent (within 1 hour)**.
 - **Transfer is not needed, and must not be arranged, as treatment delays resulting from transfer greatly increases risk of testicular infarction from torsion if present.**
 - **The exceptions:**
 - Patients presenting to the VHH Emergency Department require urgent transfer to MMC Emergency Department via consultation with ASU (General Surgery).
 - Patients less than 16 years of age require transfer to Monash Children's Hospital, Clayton, following assessment and diagnosis by Urology/ASU as outlined above.
- 6. Bedside urinalysis may be useful and can be performed but must not impede referral and review.

KEY STANDARDS, GUIDELINES OR LEGISLATION

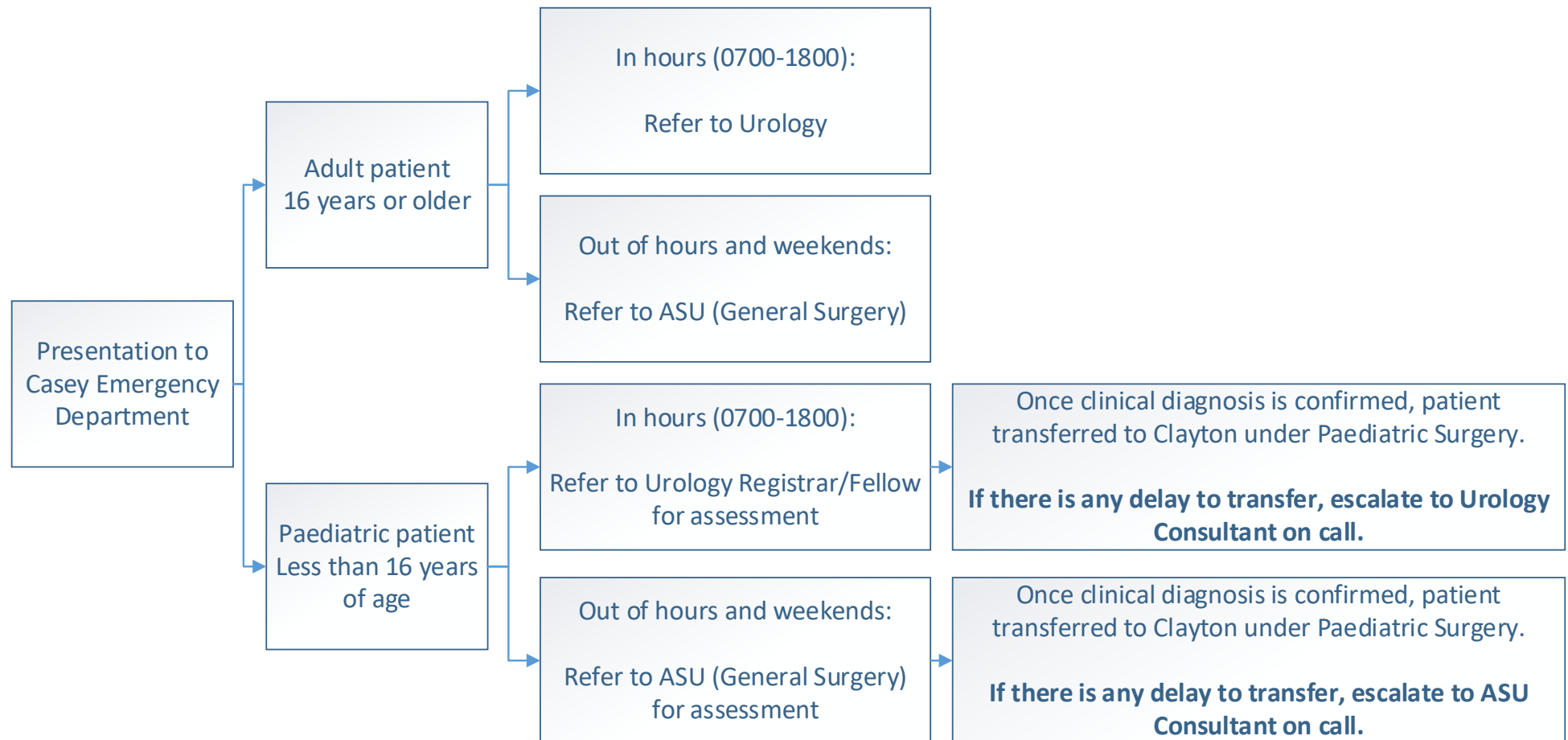
Acute Scrotal Pain and Suspected Testicular Torsion - Royal Australasian College of Surgeons Guidelines, February 2018

Document Governance		
Supporting Policy	High Quality Care (Policy)	
Executive Sponsor	Chief Operating Officer	
Program	Surgery and Interventional Services Program	
Document Author	A/Prof Paul Cashin, Service Director, General Surgery	
Consumer Review	Yes or No	Yes 29/08/2023

Management of Acute Scrotal Pain & Suspected Testicular Torsion Procedure

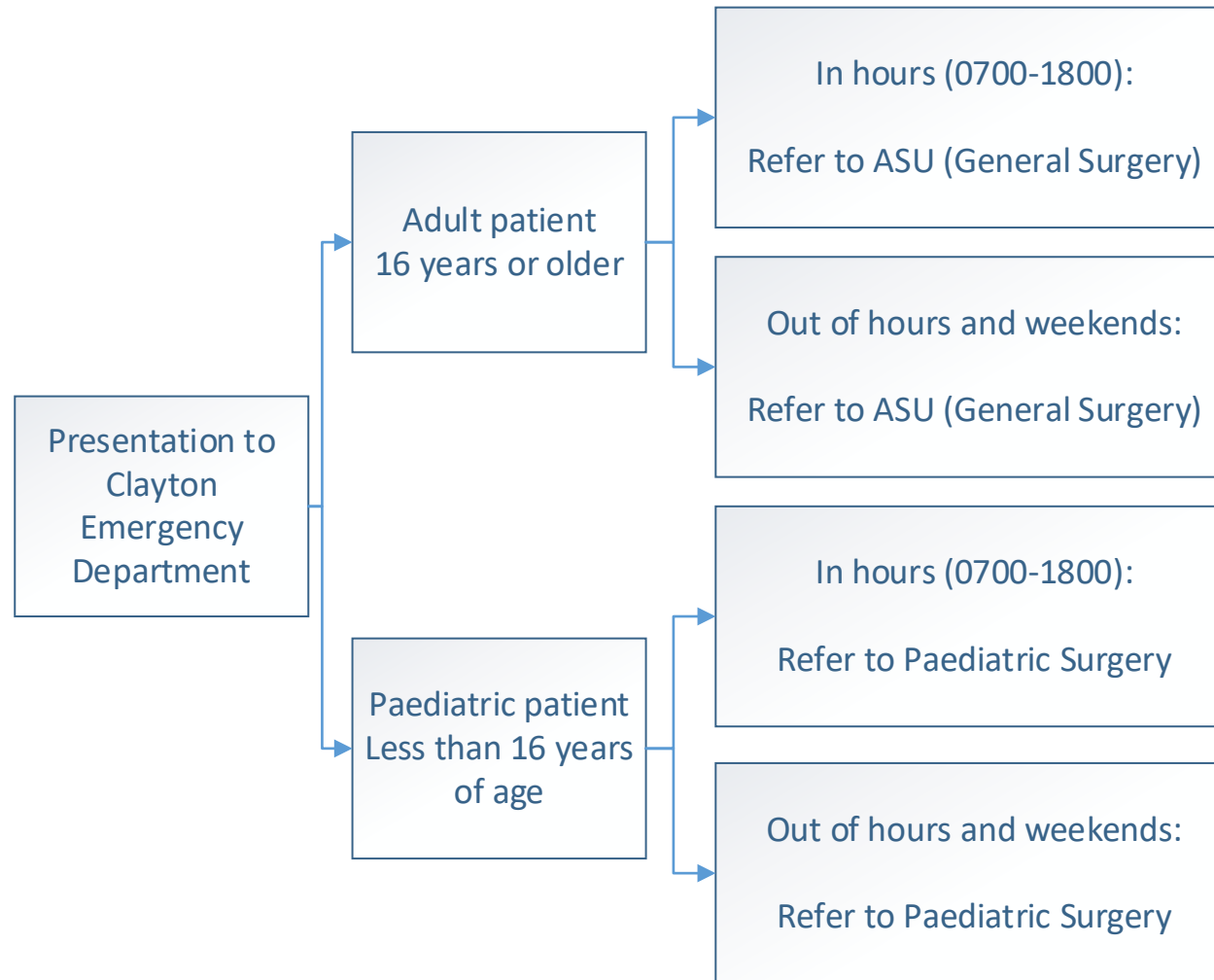
Appendix 1: Emergency Department referral flow charts

Casey Emergency Department



Management of Acute Scrotal Pain & Suspected Testicular Torsion Procedure

Clayton Emergency Department



PROMPT Doc No: SNH0033332 v11.0

Date loaded on PROMPT: 07/03/2013

Page 5 of 7

Review By: 22/10/2027

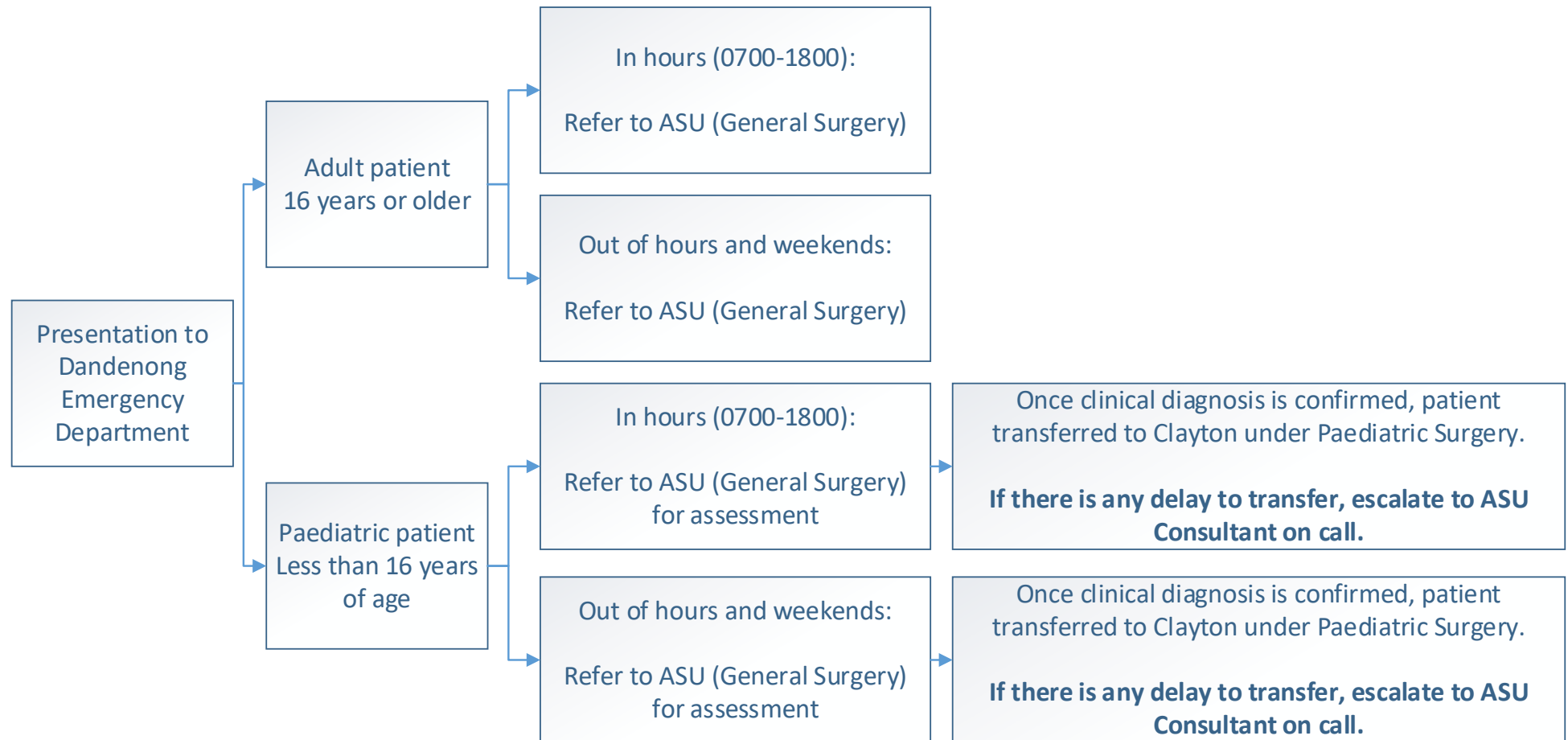
Version Changed: 22/03/2024

Document uncontrolled when
downloaded.

Last Reviewed Date: 22/03/2024

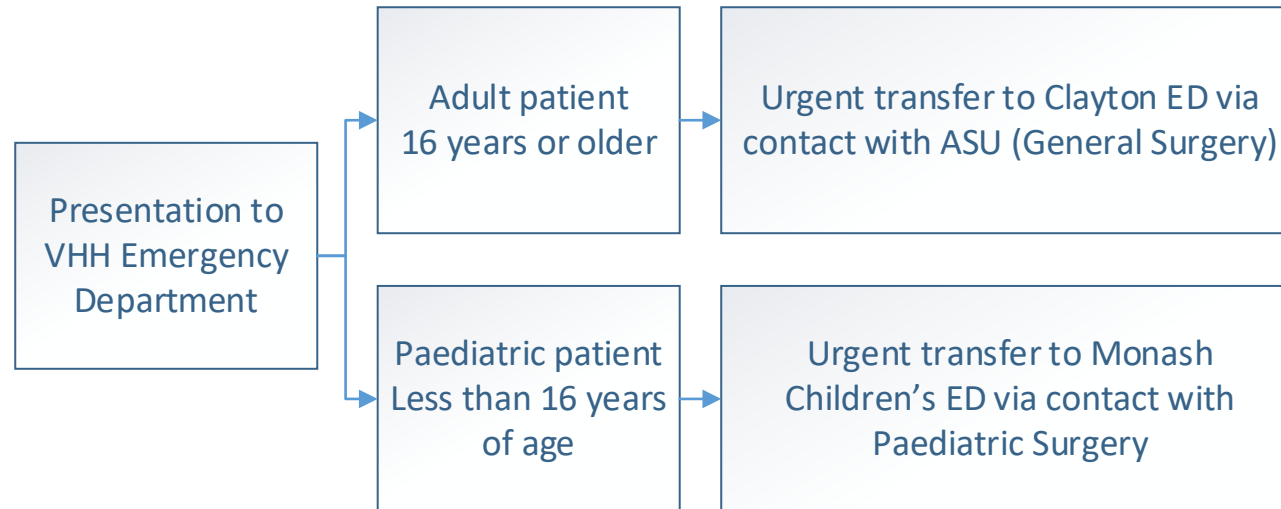
Management of Acute Scrotal Pain & Suspected Testicular Torsion Procedure

Dandenong Emergency Department



Management of Acute Scrotal Pain & Suspected Testicular Torsion Procedure

VHH Emergency Department



PROMPT Doc No: SNH0033332 v11.0		
Date loaded on PROMPT: 07/03/2013	Page 7 of 7	Review By: 22/10/2027
Version Changed: 22/03/2024	Document uncontrolled when downloaded.	Last Reviewed Date: 22/03/2024